

INTEGRATED ONCOLOGY AND PALLIATIVE CARE

An update

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OVERVIEW



- ♦ **Approach to the patient with incurable cancer**
- ♦ Contents and timeframe of Palliative Care
- ♦ Benefits of early Palliative Care
- ♦ The role of the Oncologist in Palliative Care provision
- ♦ Barriers and challenges for PC integration

APPROACH TO THE PATIENT WITH INCURABLE CANCER



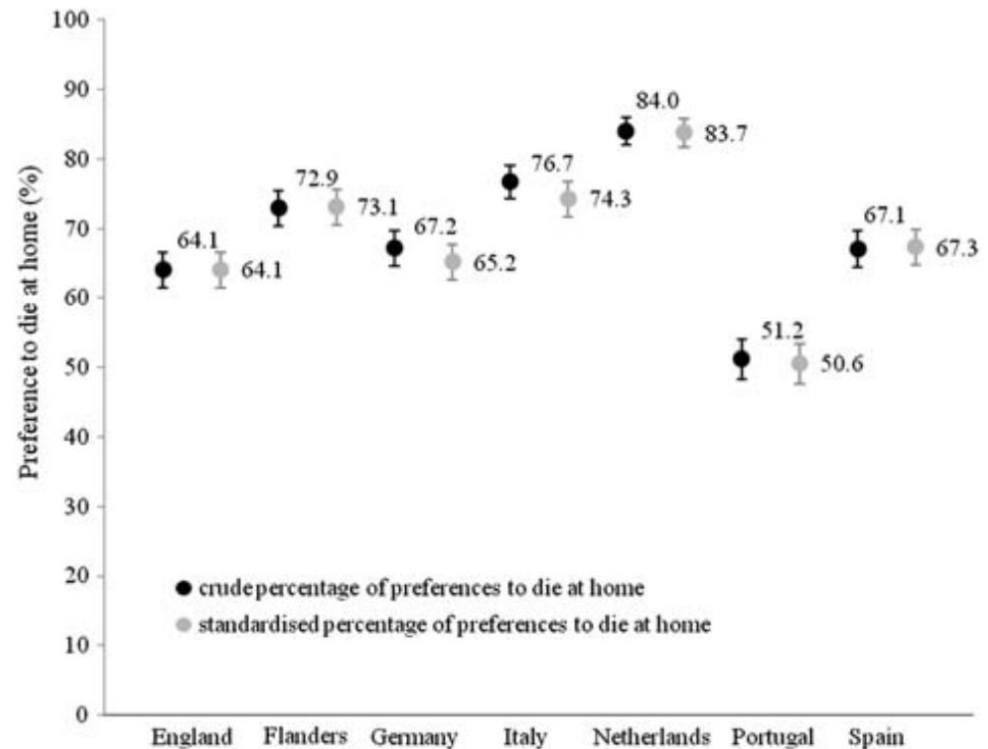
What matters to people when approaching the End-of-Life (EoL)?

- ♦ Being involved in decisions about care
- ♦ Being comfortable with controlled symptoms
- ♦ Recognition of impending death and a sense of closure
- ♦ Affirmation/value of the self, with beliefs and values honoured
- ♦ Trust in care providers
- ♦ Relationships optimised with family and friends, burden minimised to family
- ♦ Family cared for, including bereavement support
- ♦ Death in preferred place of care
- ♦ Religious prayer or meditation
- ♦ Personal affairs in order
- ♦ Leaving a legacy

APPROACH TO THE PATIENT WITH INCURABLE CANCER

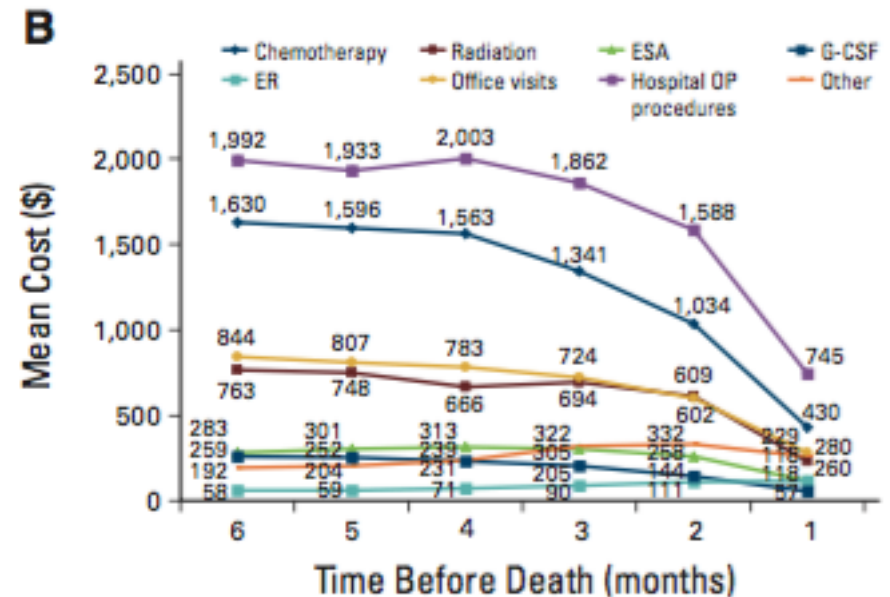
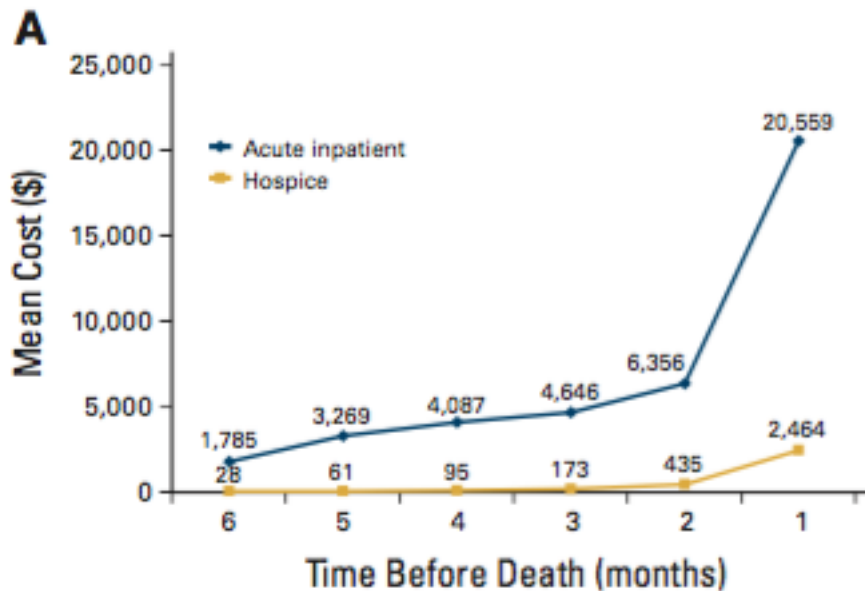
Where do people want to be taken care and die?

When faced with life-threatening illness, most people would **rather die at home**



APPROACH TO THE PATIENT WITH INCURABLE CANCER

Healthcare resources use and costs in the EoL



APPROACH TO THE PATIENT WITH INCURABLE CANCER

Total pain

“Suffering that encompasses all of a person’s physical, psychological, social, spiritual and practical struggles.”

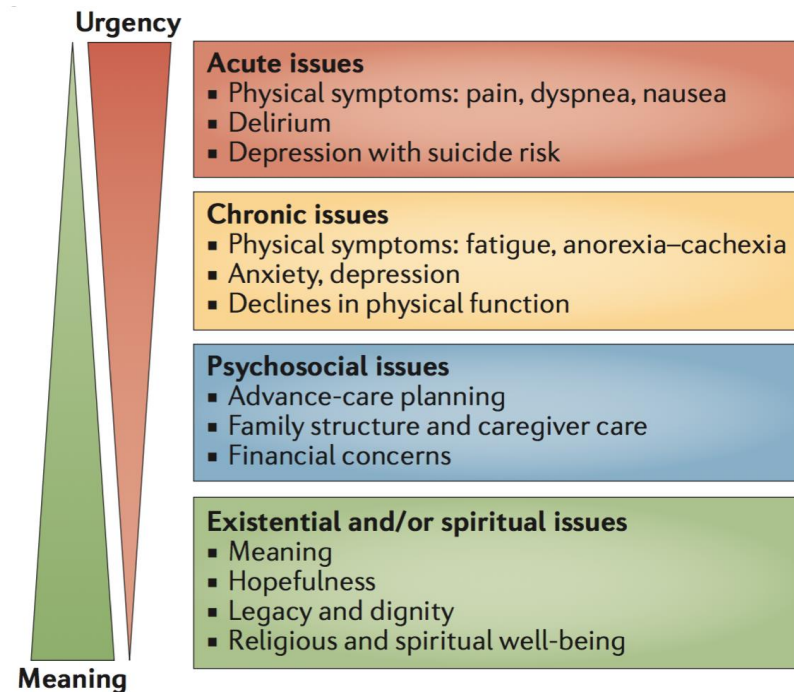
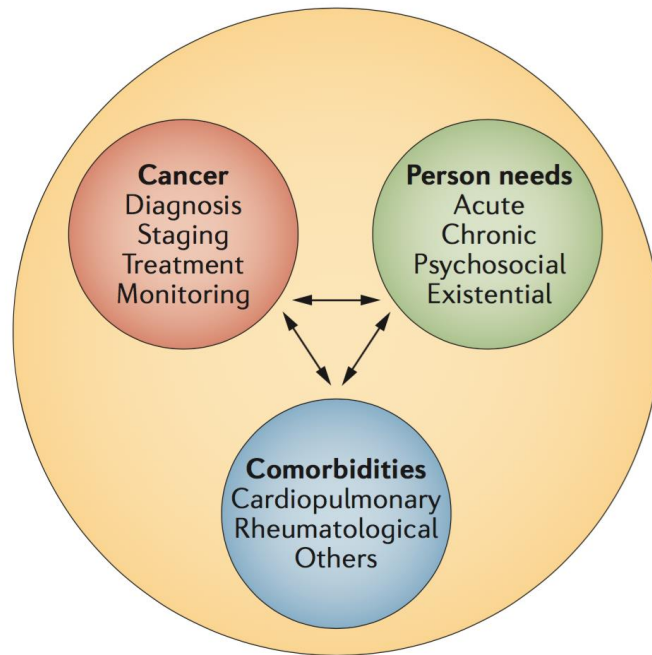
Dame Cicely Saunders



Dr Saunders with a patient at St Christopher's Hospice

APPROACH TO THE PATIENT WITH INCURABLE CANCER

Palliative care needs in oncology



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CONTENTS AND TIMEFRAME OF PALLIATIVE CARE



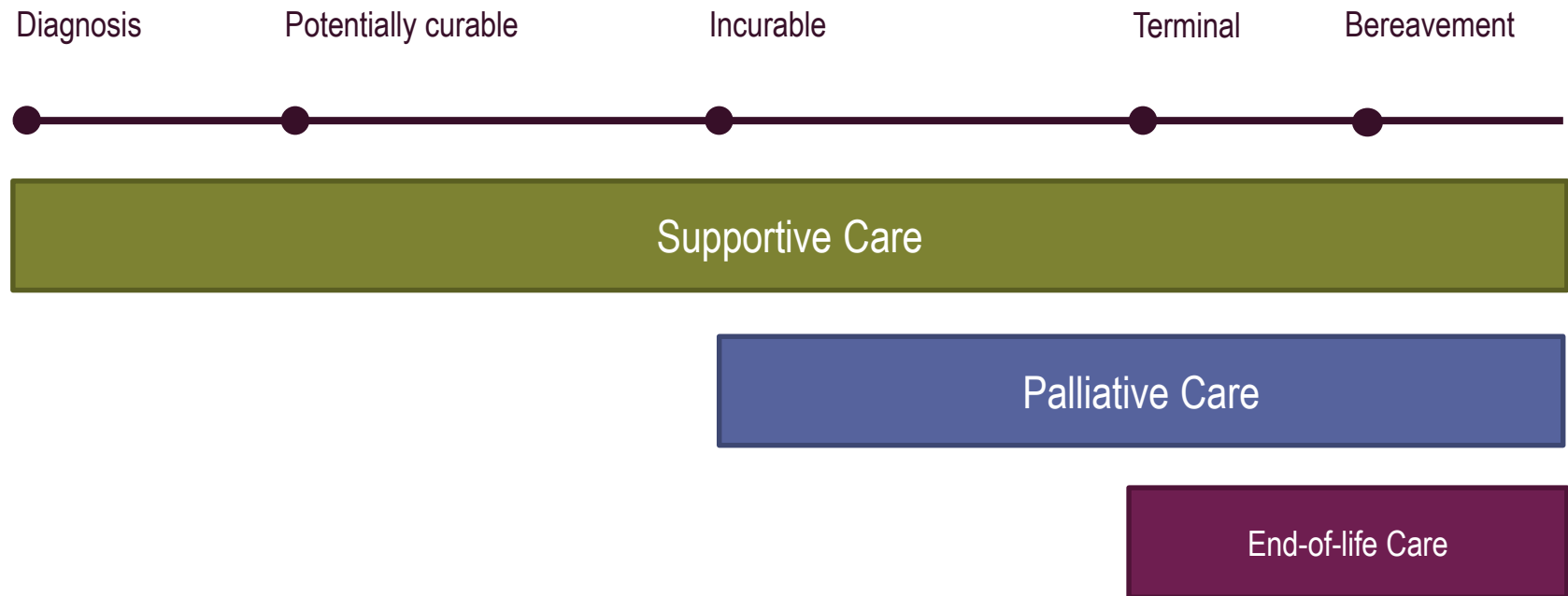
CONTENTS AND TIMEFRAME OF PALLIATIVE CARE

WHO definition (2002)

“Palliative care is an approach that improves the quality of life of patients and their families facing the problem associated with life-threatening illness, through the prevention and relief of suffering by means of early identification and impeccable assessment and treatment of pain and other problems, physical, psychosocial and spiritual.”

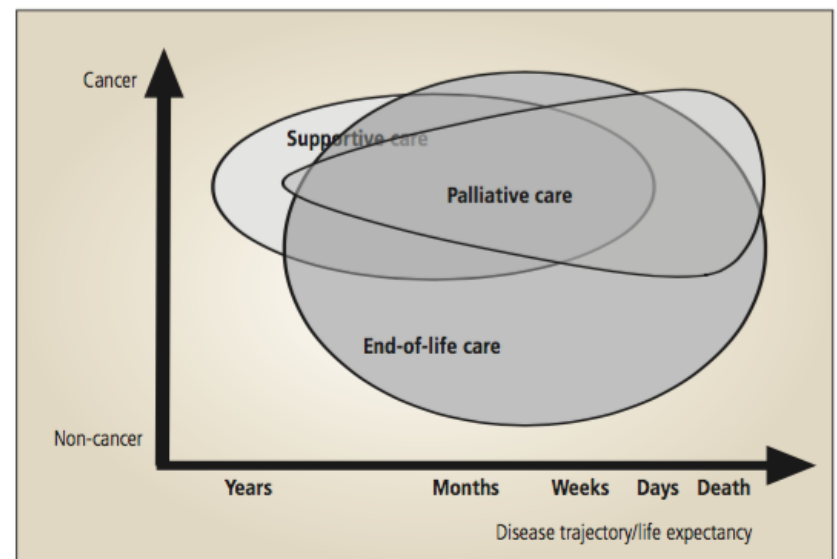
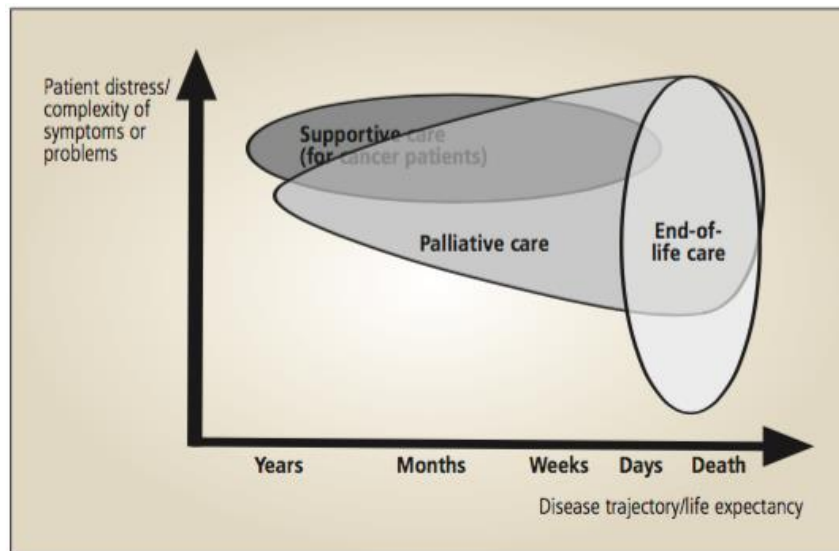
CONTENTS AND TIMEFRAME OF PALLIATIVE CARE

ESMO definition (2003)



CONTENTS AND TIMEFRAME OF PALLIATIVE CARE

EAPC white paper (2009)

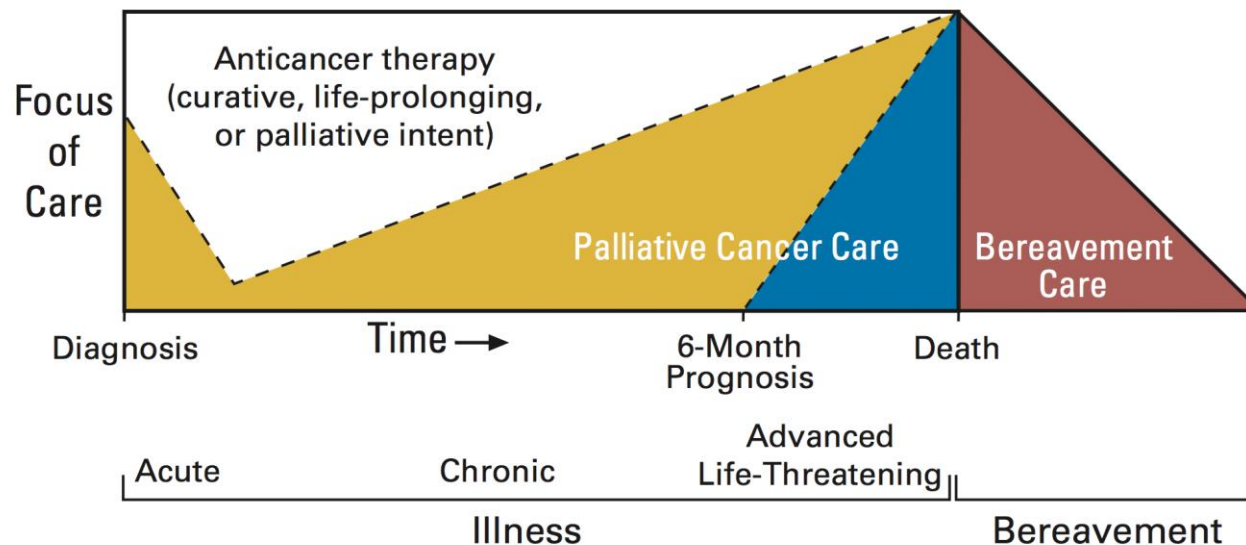


HAYWARD MEDICAL COMMUNICATIONS LTD.

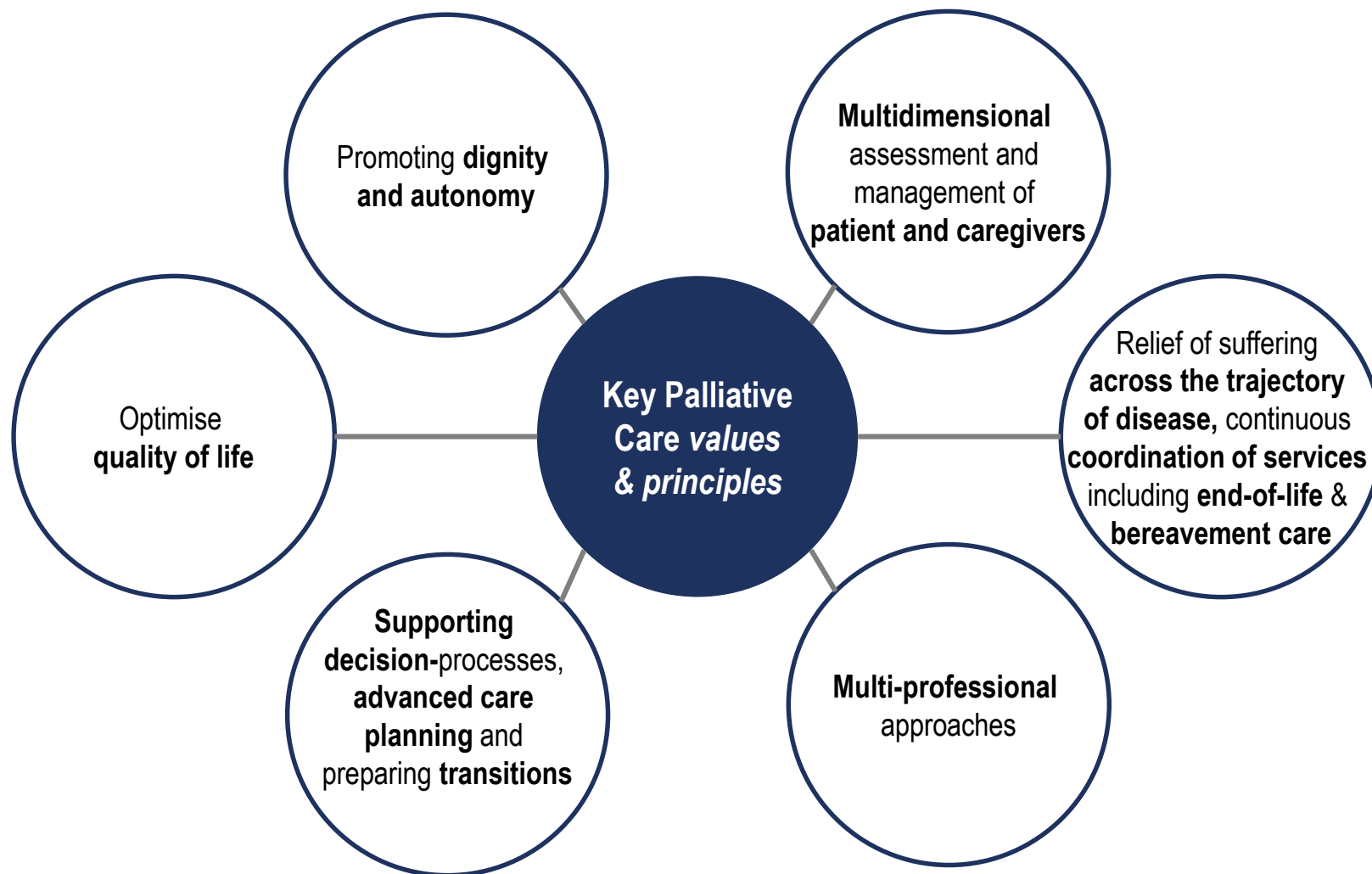
CONTENTS AND TIMEFRAME OF PALLIATIVE CARE

ASCO statement (2009) and provisional opinion (2012)

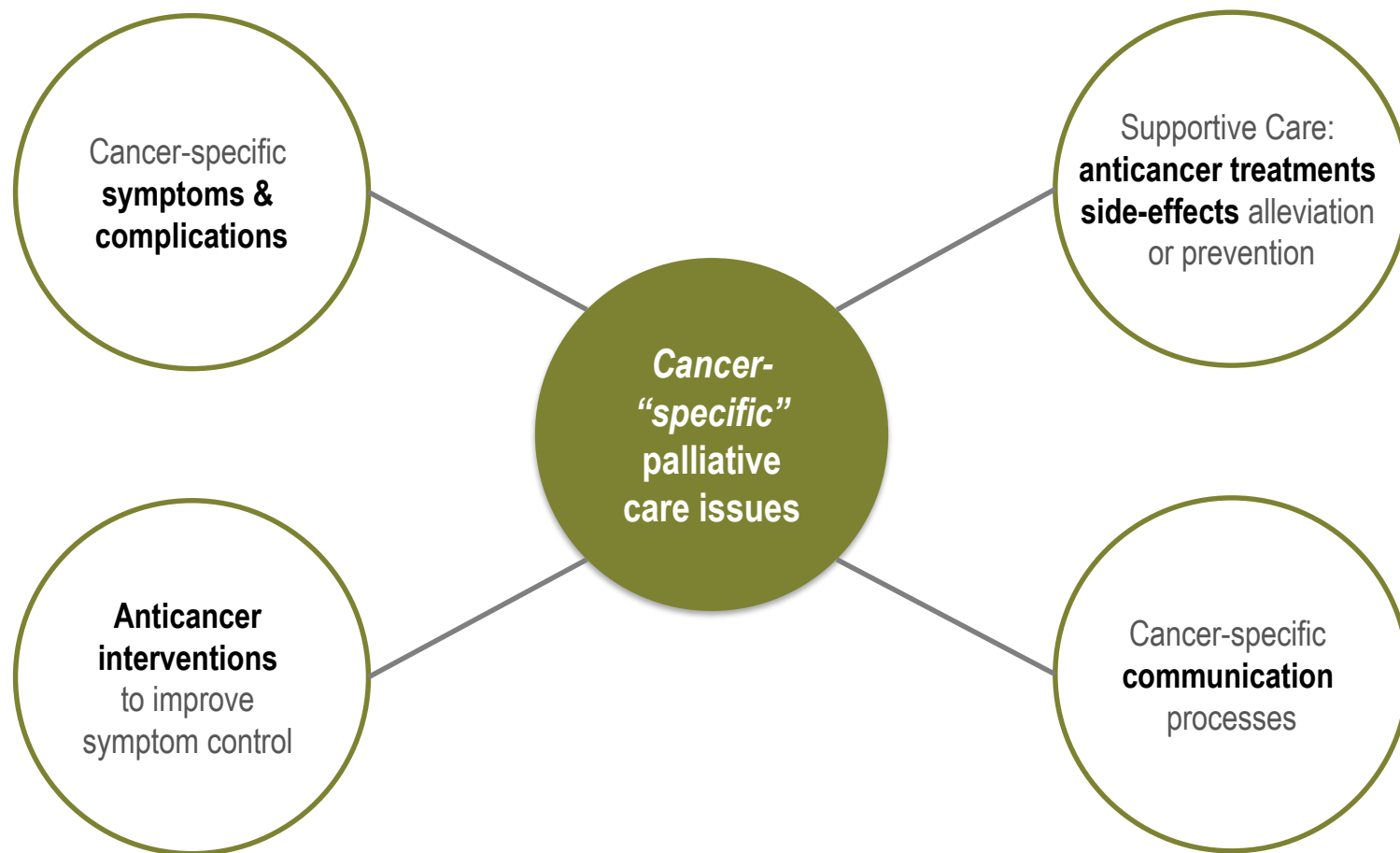
“ (...) combined standard oncology care and palliative care should be considered early in the course of illness for any patient with metastatic cancer and/or high symptom burden.”



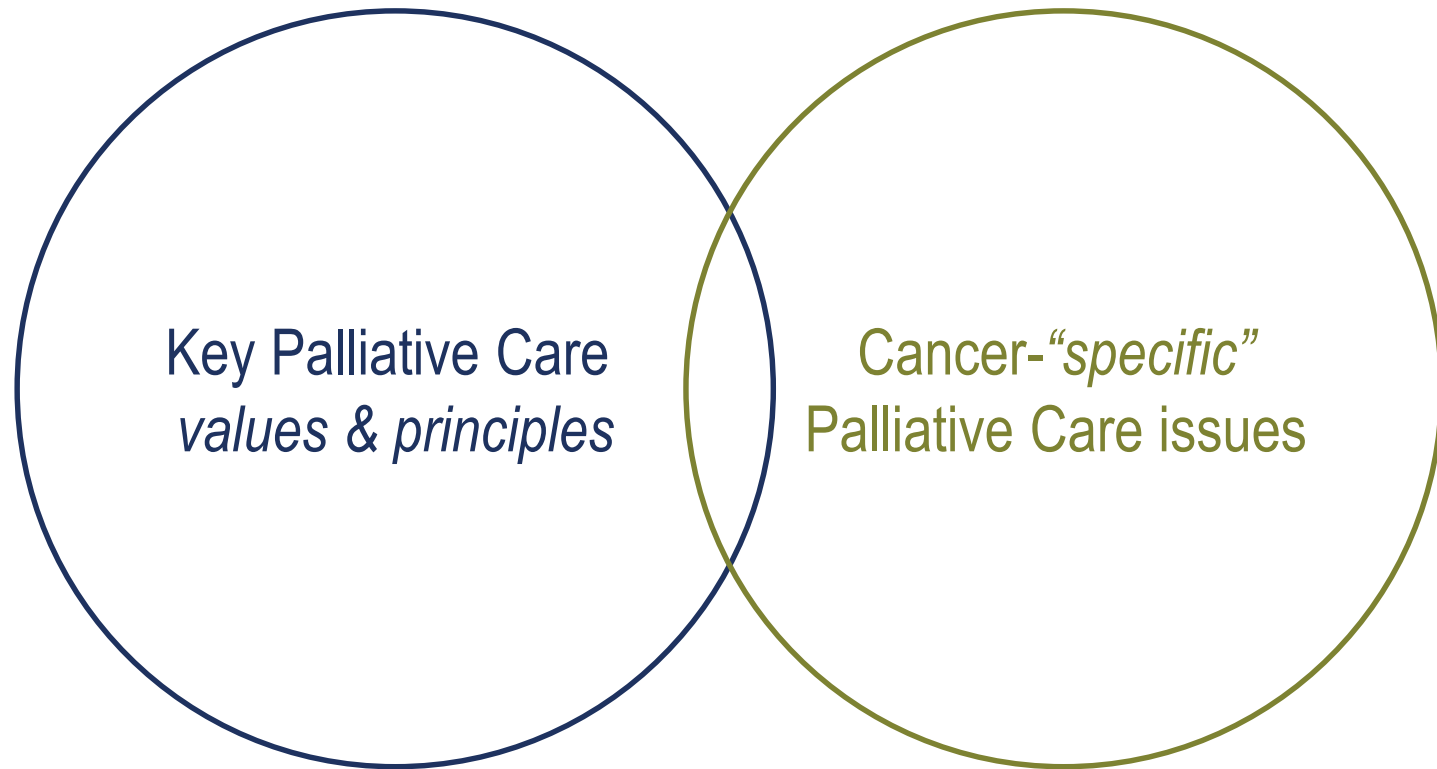
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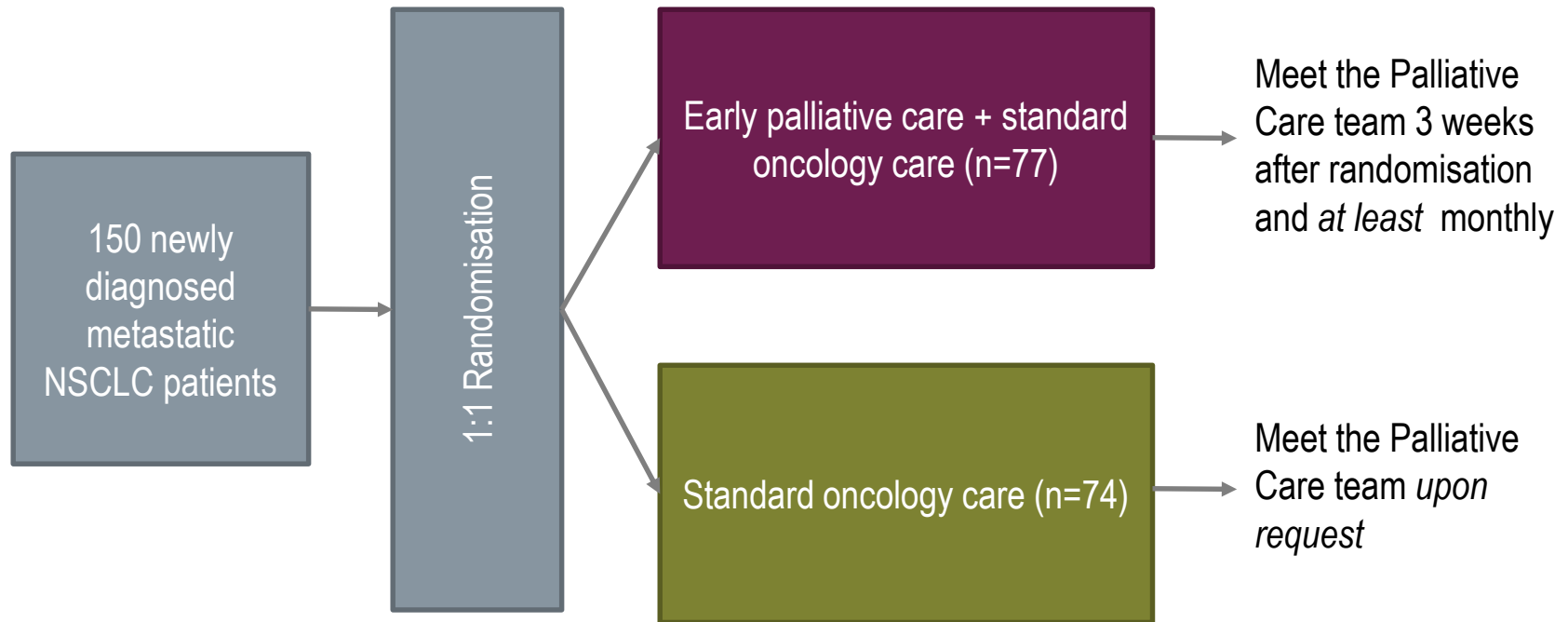


OVERVIEW



- ♦ Approach to the patient with incurable cancer
- ♦ Contents and timeframe of Palliative Care
- ♦ **Benefits of early Palliative Care**
- ♦ The role of the Oncologist in Palliative Care provision
- ♦ Barriers and challenges for PC integration in Oncology

BENEFITS OF EARLY PALLIATIVE CARE



Primary endpoint: QoL

Secondary endpoint: overall survival, mood, use of healthcare services, aggressiveness in the EoL

BENEFITS OF EARLY PALLIATIVE CARE

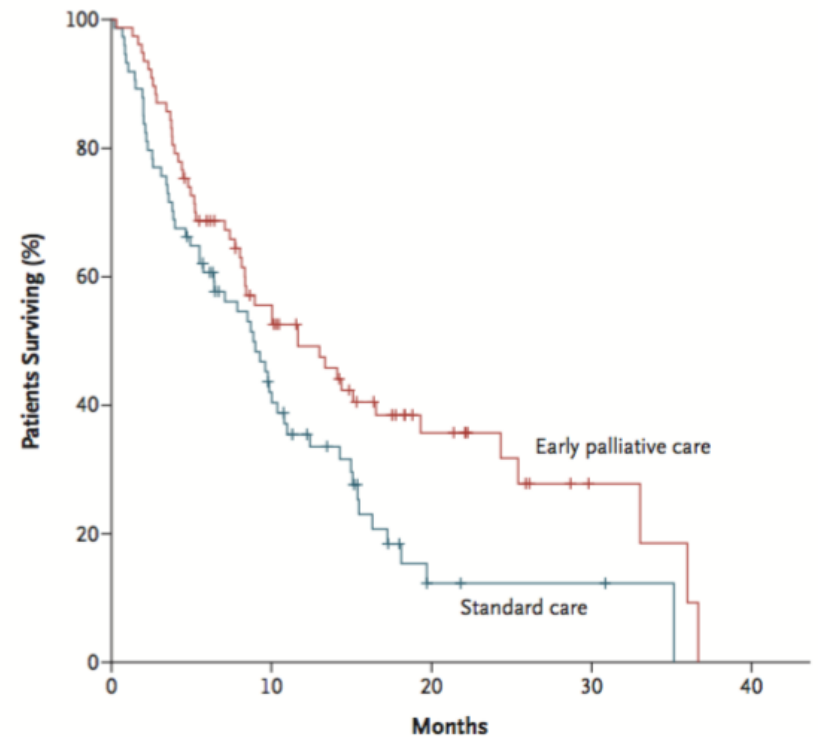
Study results:

Improved QoL in the intervention group for the total FACT-L scale, the LCS, and the Trial Outcome Index

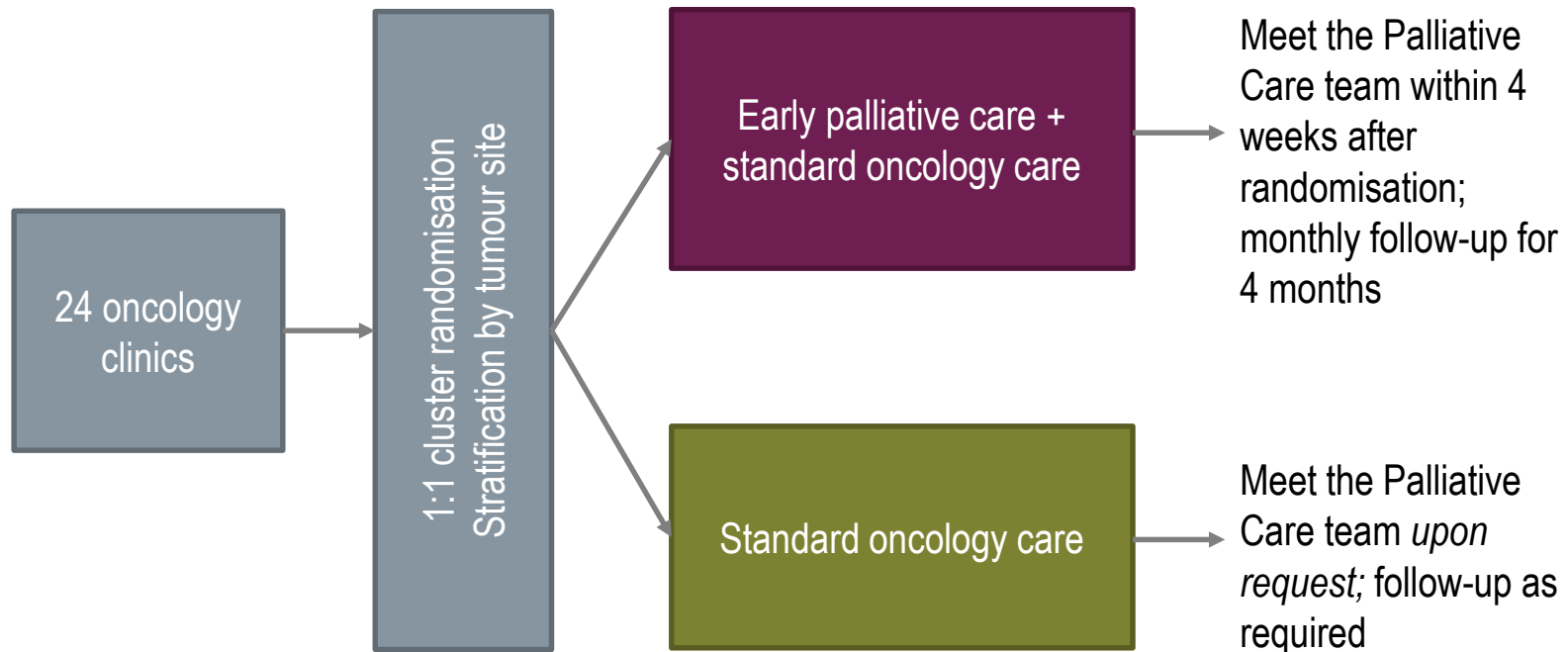
Lower depression scores in the intervention group measured by HADS and PHQ-9

More aggressive end-of-life care in the control group (54% vs. 33%, $p = 0.05$)

Less advanced care planning documentation in the control group (28% vs. 53%, $p = 0.05$)



BENEFITS OF EARLY PALLIATIVE CARE



Primary endpoint: QoL

Secondary endpoint: symptom control, communication with healthcare providers, patient and caregiver satisfaction with care and caregiver quality of life

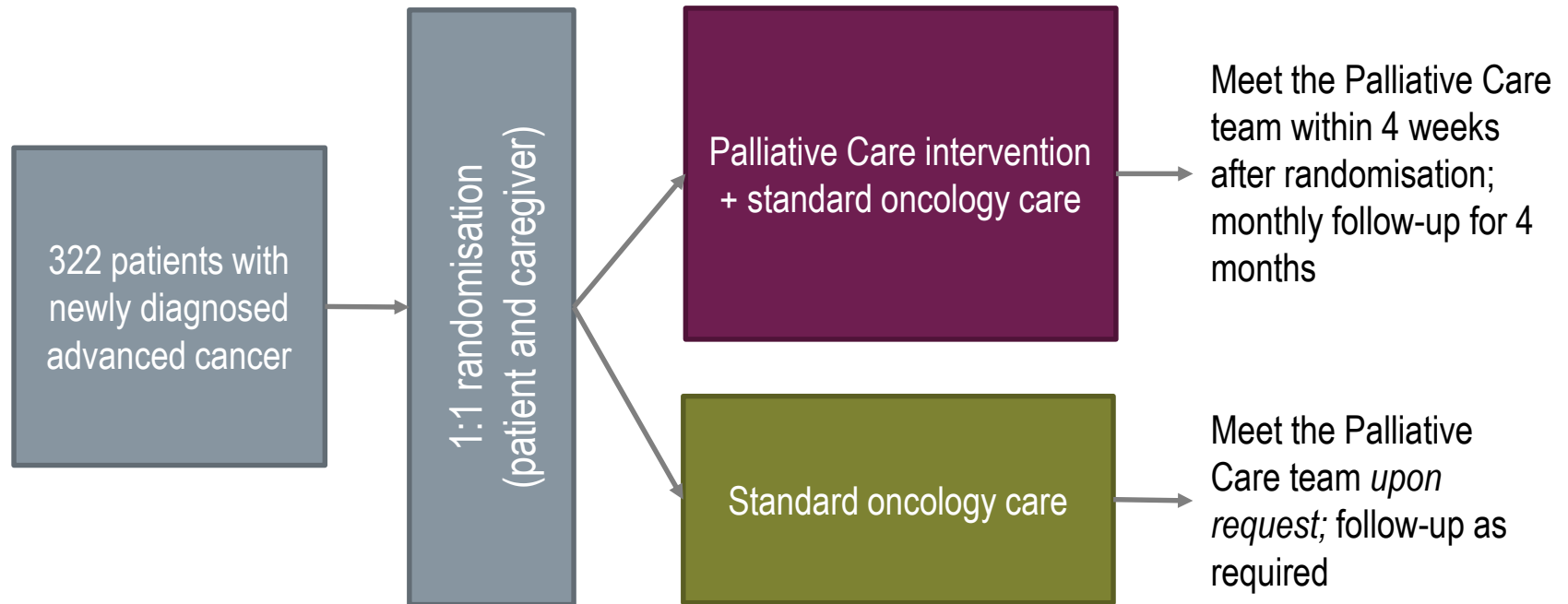
BENEFITS OF EARLY PALLIATIVE CARE



- ♦ Non-significant difference in change score for FACIT-Sp between intervention and control groups
- ♦ Significant difference in QUAL-E and FAMCARE-P16 favouring early PC group
- ♦ No difference in ESAS or CARES-MIS between intervention and control groups

BENEFITS OF EARLY PALLIATIVE CARE

ENABLE II trial



Primary endpoints: patient-reported quality of life (QoL), symptom intensity, and resource use

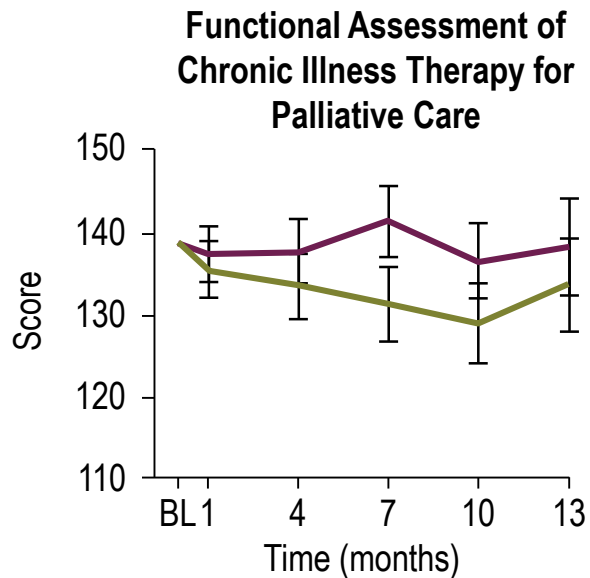
Secondary endpoint: mood

BENEFITS OF EARLY PALLIATIVE CARE

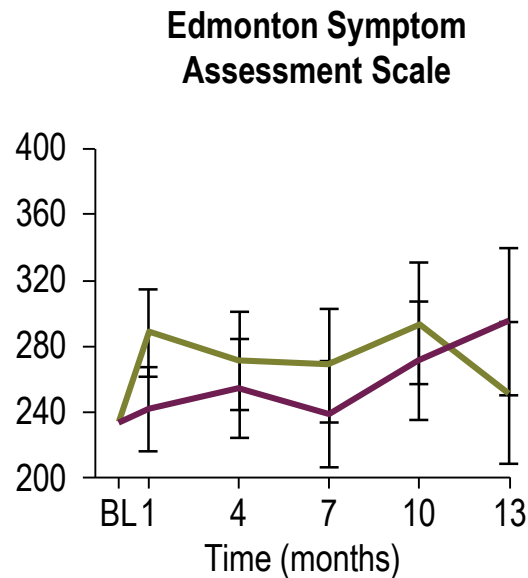
Patient outcomes



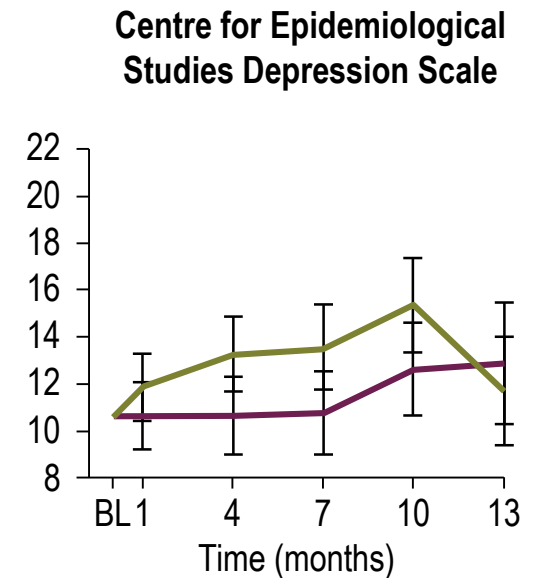
QoL



Symptom control



Mood



Patients, No.

Intervention	143	108	69	59	48	27
Usual care	130	97	74	54	44	31

145	109	73	62	48	28
134	100	76	54	45	31

140	102	72	60	47	26
128	98	76	54	44	31

BENEFITS OF EARLY PALLIATIVE CARE

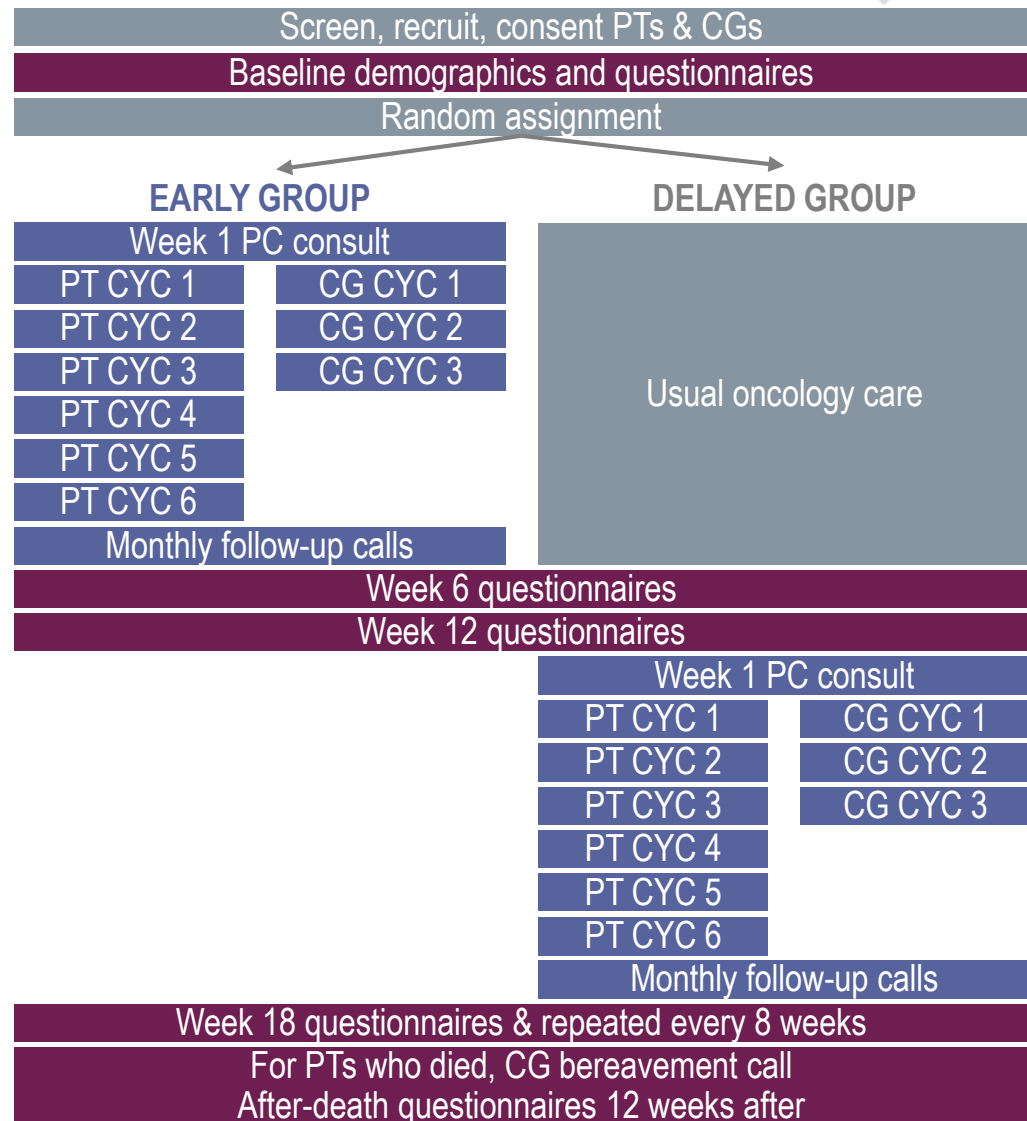
ENABLE III trial

Patients' outcomes:

- QoL
- Symptom control
- Mood
- 1-year survival
- Resources use
- Place of death

Caregivers' outcomes:

- QoL
- Depressed mood
- Burden



BENEFITS OF EARLY PALLIATIVE CARE

ENABLE III trial



Patients' outcomes:

- ◆ No differences in PROs (HQoL, mood, symptom control) at 3 months from enrolment
- ◆ 15% difference in 1-year survival for early PC group
- ◆ No differences in resources use (hospital admission, emergency visits or ICU admission)
- ◆ No differences in the place of death

Caregivers' outcomes:

- ◆ Lower depression scores in the early PC group (mean difference, 3.4; SE, 1.5; d .32; P .02)
- ◆ No differences in QoL (mean difference, 2; SE, 2.3; d .13; P .39)
- ◆ No differences in demand burden (objective burden, stress burden and demand burden)

BENEFITS OF EARLY PALLIATIVE CARE



Early Palliative Care may **improve**:

- ♦ Patients' and caregivers QoL and preparation of the EoL period
- ♦ Symptom control (physical and psycho-spiritual distress)
- ♦ Survival
- ♦ The likelihood of respecting patients' preferences

Early Palliative Care may **reduce**:

- ♦ Use of healthcare resources (and indirectly costs) in the EoL
- ♦ Aggressive EoL care

OVERVIEW

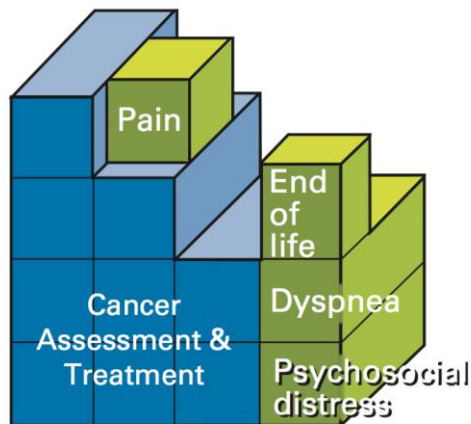


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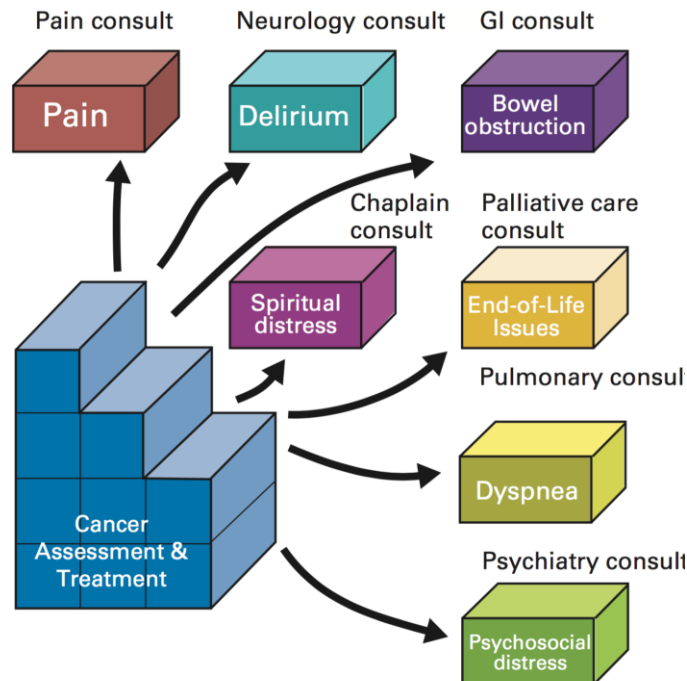
THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

Models of Palliative Care integration

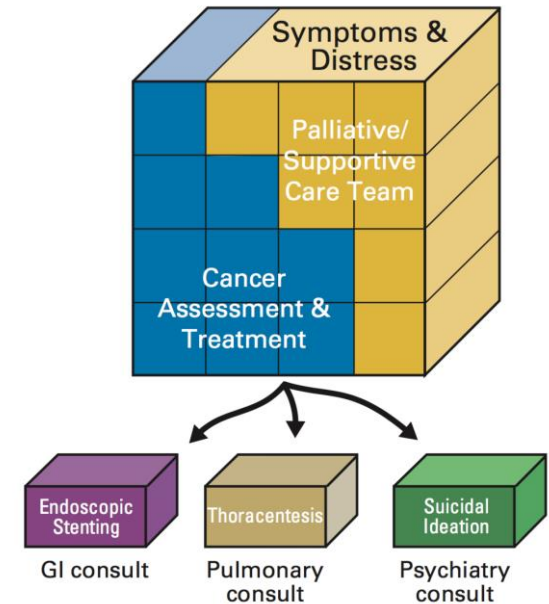
Solo Practice Model



Congress Practice Model



Integrated Care Model



THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

Core competencies in Palliative Care (EAPC 2013)

1. Apply the core constituents of palliative care in the setting where patients and families are based
2. Enhance physical comfort throughout patients' disease trajectories
3. Meet patients' psychological needs
4. Meet patients' social needs
5. Meet patients' spiritual needs
6. Respond to the needs of family caregivers in relation to short-, medium- and long-term patient care goals
7. Respond to the challenges of clinical and ethical decision-making in palliative care
8. Practice comprehensive care co-ordination and interdisciplinary teamwork across all settings where palliative care is offered
9. Develop interpersonal and communication skills appropriate to palliative care
10. Practice self-awareness and undergo continuing professional development

THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

Skills (ESMO / ASCO 2016)



Supportive Care

- Preventive and therapeutic strategies for common side effects of therapy
- Management of all cancer-related symptoms of any tumour entity;
- Counseling on side effects of therapy;
- Management of oncological emergencies.

Palliative Care

- Recognizing patients in need for Palliative Care and triggers / criteria for referral to SPC
- Pathophysiology of common cancer syndromes and pharmacology of medications used to treat main symptoms and to prevent toxicities
- Plan and coordinate care in a MDT
- Comprehensive assessment of patients with complex symptoms
- Compassionated communication with patients and families
- Personal plan to self care and to prevent burnout

End-of-Life Care

- Disclosing prognosis in order to prepare patients to the dying process
- Communicate the benefits / limitations of anti-cancer therapies
- Run effective family care conferences
- Establish patient preferences
- Identify refractory symptoms that require SPC, namely palliative sedation
- Coordinate referrals to home care, nursing homes and hospice

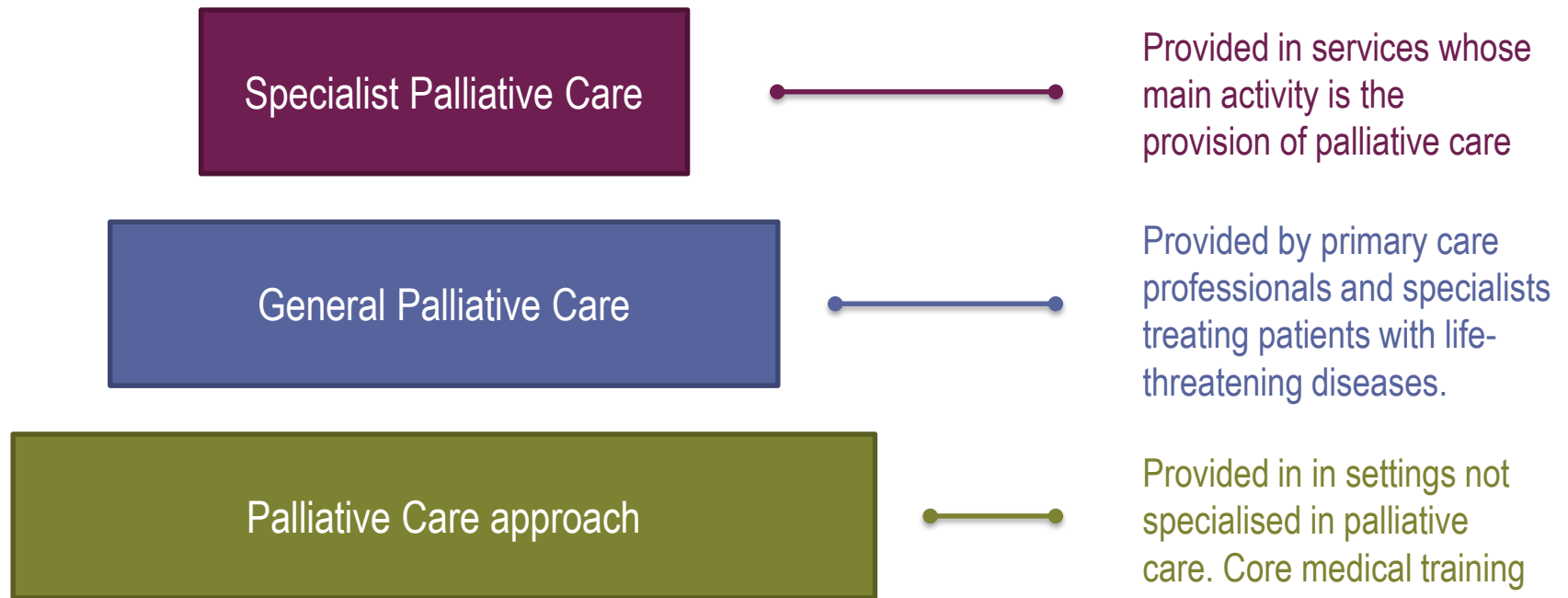
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High-quality palliative care in oncology practice (ASCO / AAHPM guidance statement 2015)

1. Symptom assessment and management
2. Psychosocial assessment and management
3. Spiritual and cultural assessment and management
4. Communication and shared decision-making
5. Advance care planning, including ethical and legal issues
6. Coordination and continuity of care
7. Appropriate palliative care and hospice referral
8. Caregiver support (family/caregiver and staff)
9. End-of-life care

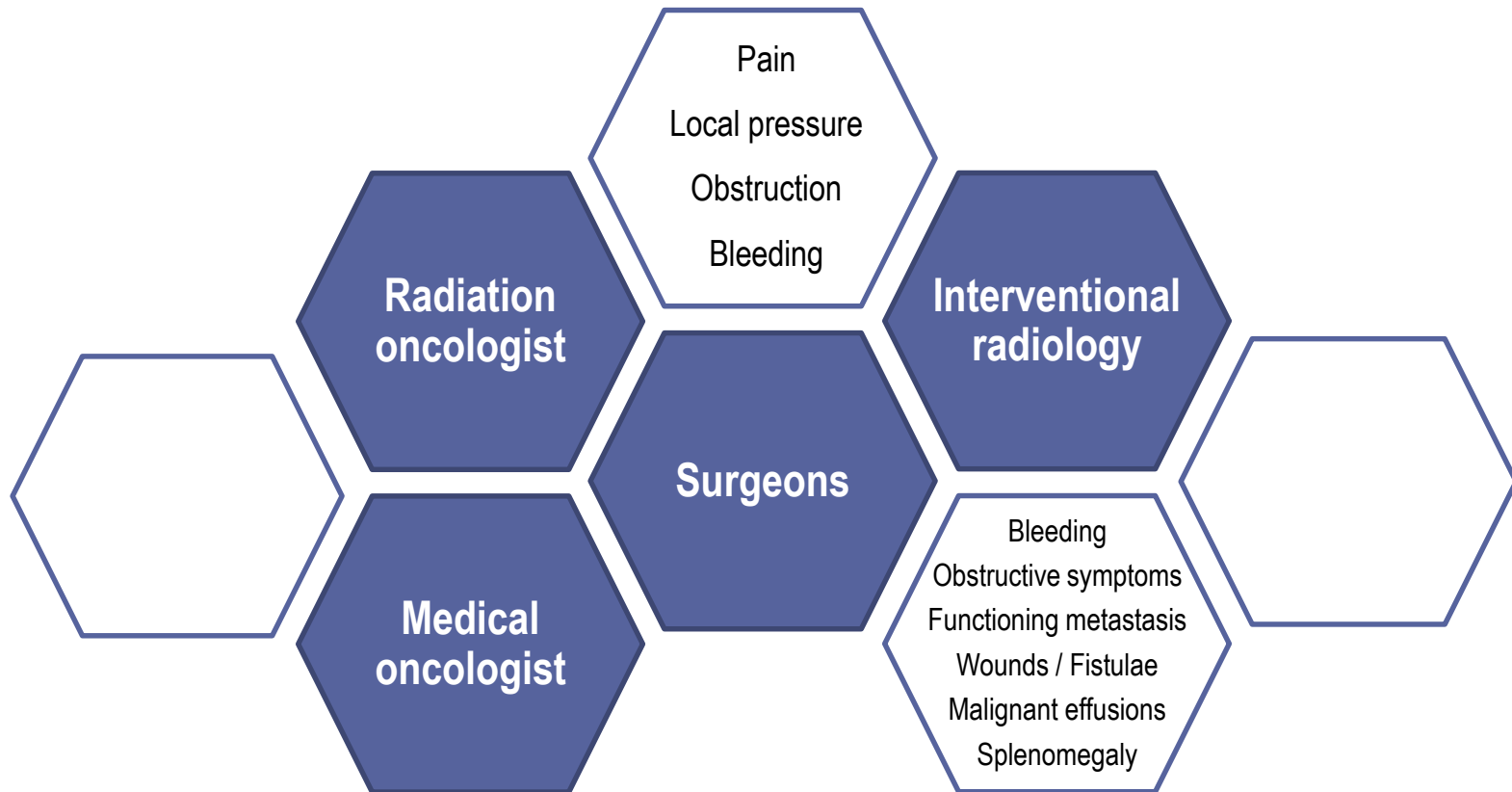
THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

Levels of Palliative Care provision



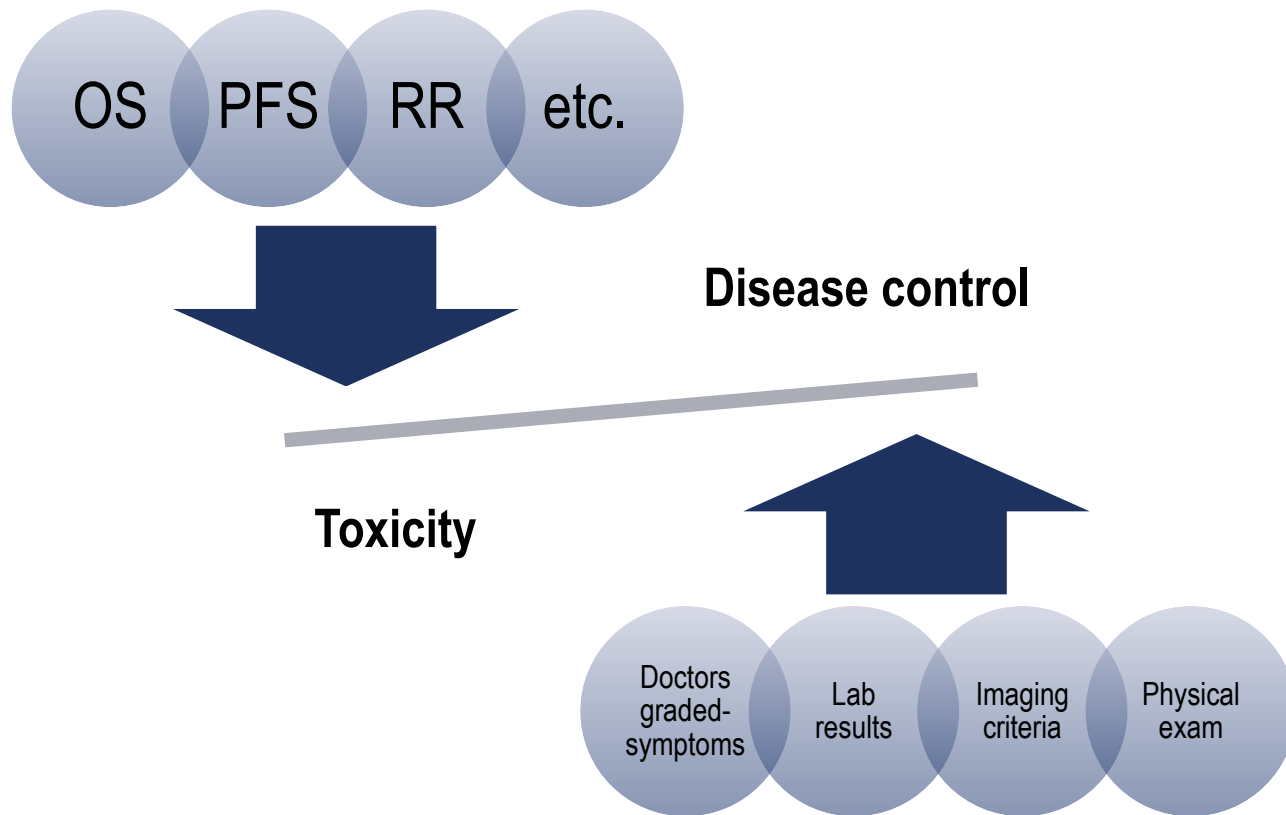
THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

Disease-modifying management in advanced and incurable cancer



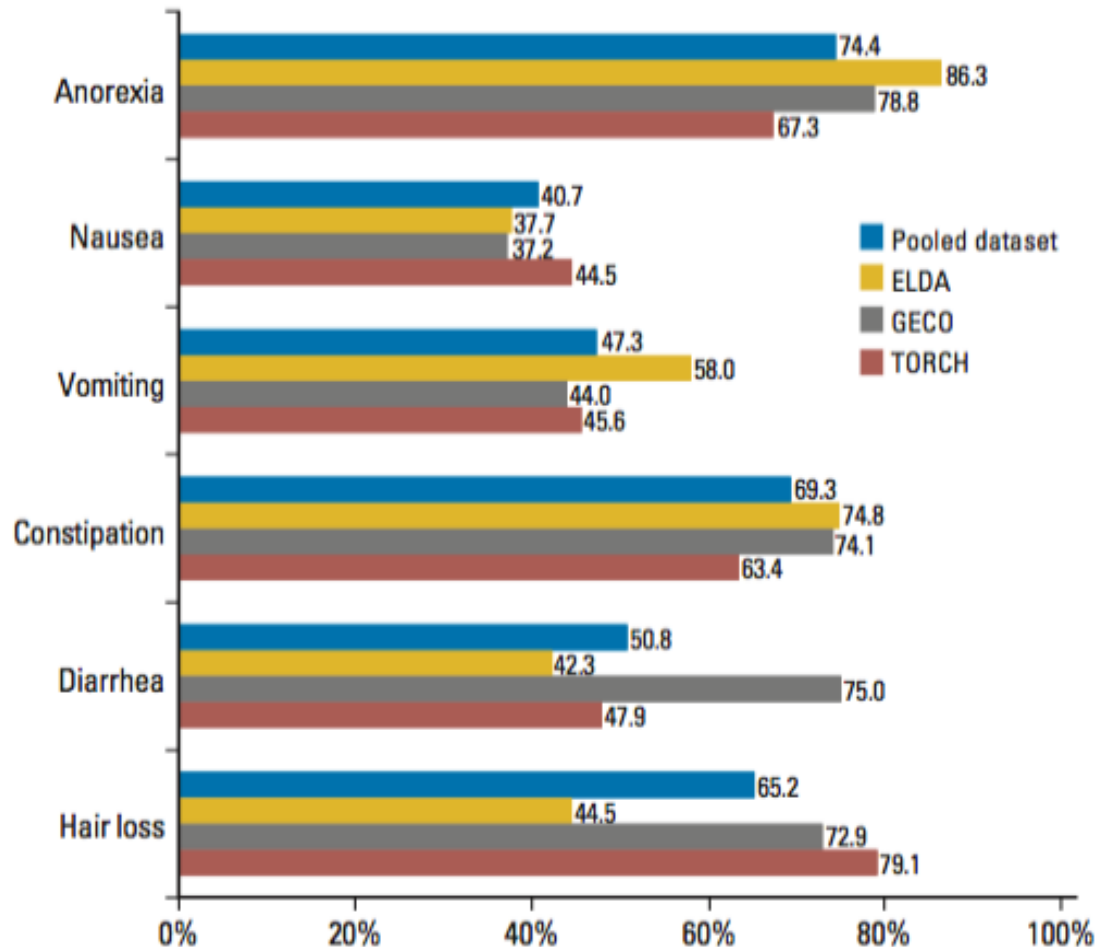
THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

Treatment-related decision-making process in advanced cancer



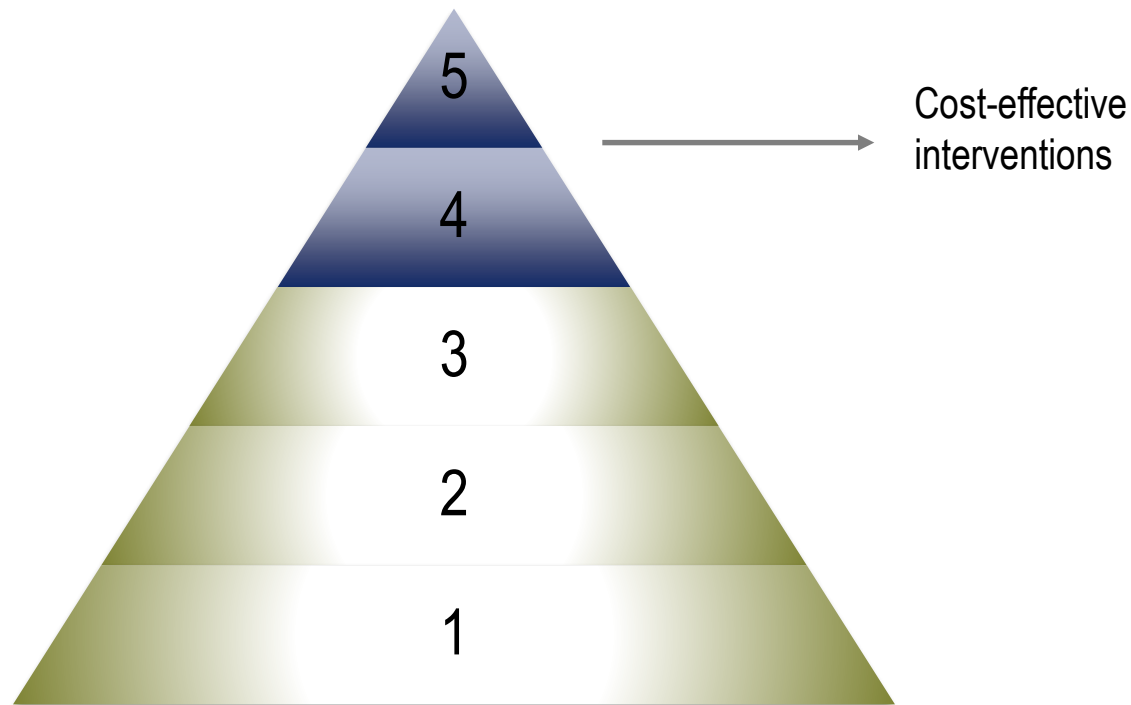
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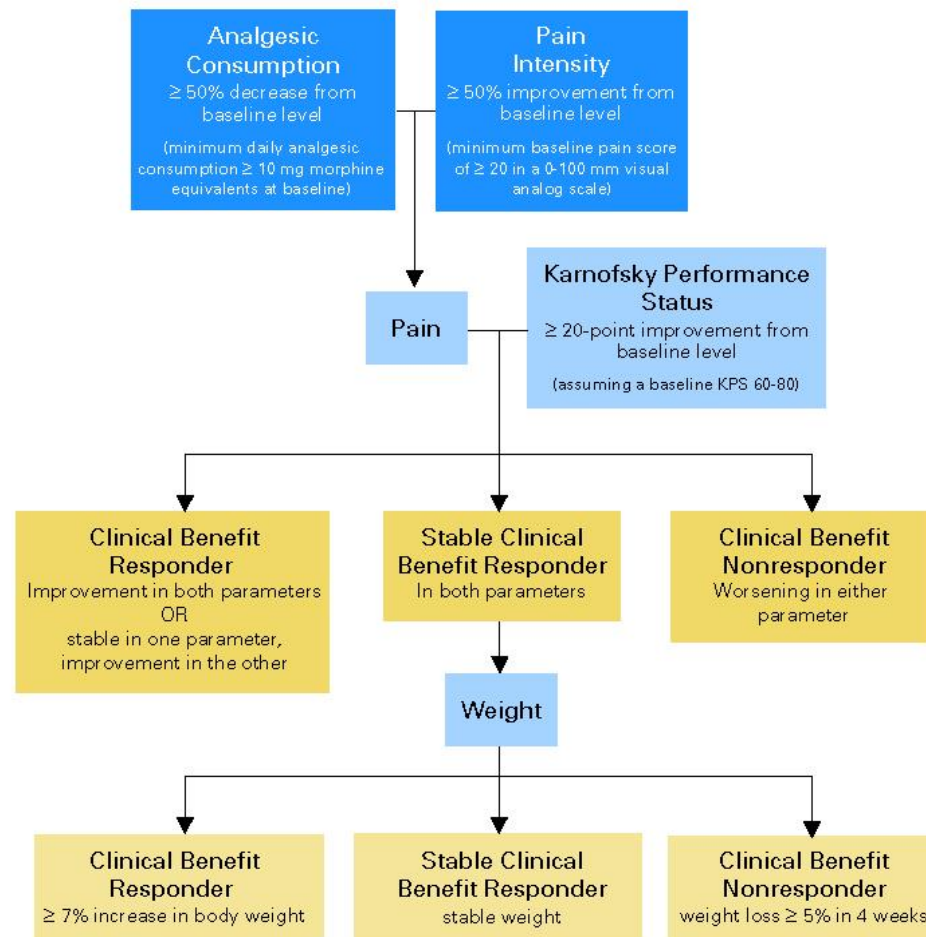
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Magnitude of Clinical Benefit Scale (ESMO 2015)



THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

Anti-cancer treatment to alleviate or prevent cancer-related suffering



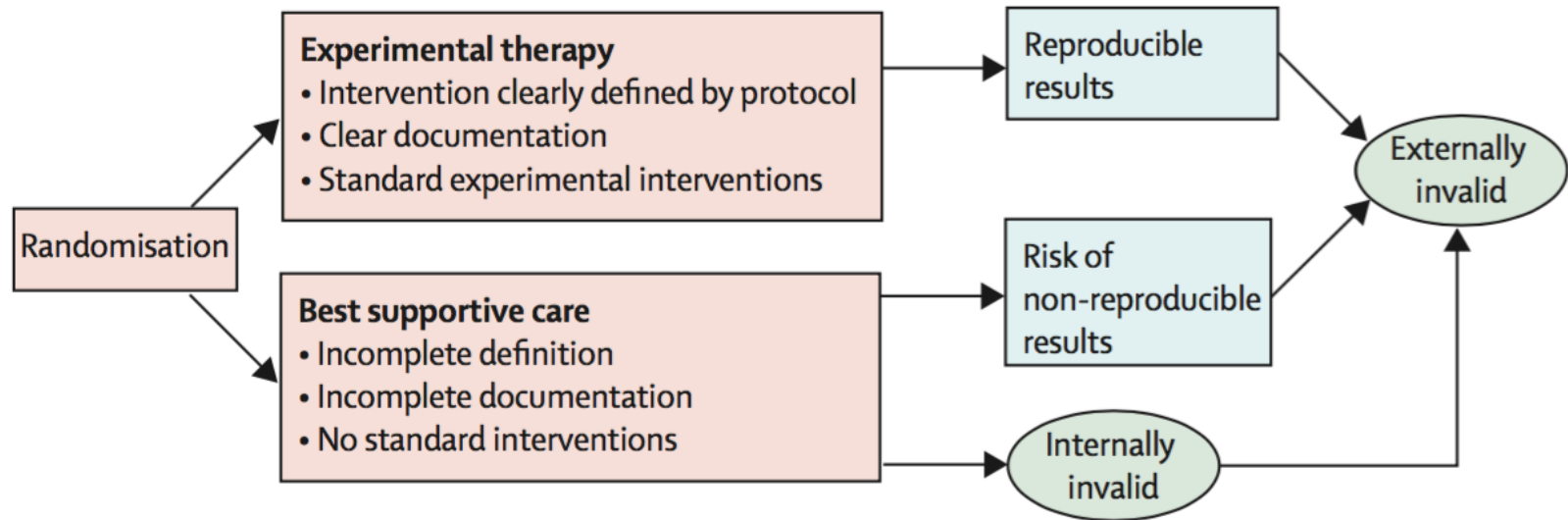
THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

Anti-cancer treatment to alleviate or prevent cancer-related suffering

	N	Improved	Controlled	Prevented	Palliated*	p value†
Lethargy						
ASC	110	4 (4%)	33 (35%)	18 (19%)	55 (50%)	
ASC+MVP	122	11 (9%)	22 (18%)	22 (18%)	55 (45%)	0.54
ASC+V	119	8 (7%)	17 (14%)	18 (15%)	43 (36%)	0.047
Chest pain						
ASC	111	8 (7%)	23 (21%)	23 (21%)	54 (49%)	
ASC+MVP	122	13 (11%)	37 (30%)	35 (29%)	85 (70%)	0.0017
ASC+V	117	12 (10%)	29 (25%)	32 (27%)	73 (62%)	0.051
Sweating						
ASC	111	5 (5%)	21 (19%)	46 (41%)	72 (65%)	
ASC+MVP	122	11 (9%)	21 (17%)	66 (54%)	98 (80%)	0.012
ASC+V	117	7 (6%)	16 (14%)	59 (50%)	82 (70%)	0.48
Anorexia						
ASC	111	8 (7%)	16 (14%)	48 (43%)	72 (65%)	
ASC+MVP	122	7 (6%)	18 (15%)	61 (50%)	86 (70%)	0.44
ASC+V	117	8 (7%)	14 (12%)	55 (47%)	77 (66%)	0.99

THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

“Best supportive care” arm in clinical trials



Four domains for delivery BSC in clinical trials:

- ◆ Multidisciplinary care, documentation, symptom assessment and symptom management

THE ROLE OF ONCOLOGISTS IN PALLIATIVE CARE PROVISION

“Best supportive care” arm in clinical trials

Domain assessed	Yes
Conformance to BSC consensus standards in clinical trials	
Standardized BSC across sites if multisite	0%
BSC delivery standardized	6%
Clear description of BSC	33%
Symptom assessment at regular intervals	61%
Symptom assessment with valid tools	44%
Symptom assessment identical both arms	33%
Symptom management is evidence based	0%
Reported educating patients about symptom management	0%
Reported educating patients about goals of therapy	0%
Provided access to palliative specialists	0%
Provided access to support services	11%
Education about goals, importance of symptom assessment/management	0%
Conformance to consort checklist for RCT's	
Eligibility criteria	100%
Documentation of settings	11%
Interventions described sufficiently	0%
Generalizable results (must have met all three of the above CONSORT criteria)	0%
Abbreviations: BSC = best supportive care; RCT = randomized controlled trial.	

Nipp RD, *et al.* Best supportive care in clinical trials: review of the inconsistency in control arm design.
<http://www.nature.com/bjc/journal/v113/n1/full/bjc2015192a.html>. Br J Cancer 2015;113:6–11,
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OVERVIEW



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- ♦ **Barriers and challenges for PC integration in Oncology**

BARRIERS AND CHALLENGES FOR PC INTEGRATION



Attitudes of medical oncologists towards Palliative Care

ESMO Taskforce on Palliative and Supportive Care survey (2003)

The majority of the oncologists agreed that:

- ♦ All advanced cancer patients should receive concurrent palliative care, even if they are receiving antitumour therapies
- ♦ All cancer centres should have a palliative care service
- ♦ Medical oncologists should coordinate the care of cancer patients at all stages of disease, including end-of-life care
- ♦ Medical oncologists should be expert in the management of the physical and psychological symptoms of advanced cancer
- ♦ I am usually successful in managing my patients' pain

There were polarised answers:

- ♦ I received good training in palliative care during my oncology fellowship
- ♦ I feel emotionally burned out by having to deal with too many deaths
- ♦ Most medical oncologists I know are expert in the management of the physical and psychological symptoms of advanced cancer
- ♦ A palliative care specialist is the best person to coordinate the palliative care of patients with advanced cancer
- ♦ Palliative care physicians do not have enough understanding of oncology to counsel patients with advanced cancer regarding their treatment options

BARRIERS AND CHALLENGES FOR PC INTEGRATION

Oncologists' factors that may influence referral

Oncologists' views of palliative care

- ♦ Alternative to anti-cancer therapy
- ♦ Complementary to anti-cancer therapy
- ♦ Evolving views of palliative care

Oncologists' self-defined professional role

- ♦ Includes providing palliative care
- ♦ Focused on anti-cancer therapy

Knowledge of available services of Palliative Care

BARRIERS AND CHALLENGES FOR PC INTEGRATION



Possible barriers to integration of Palliative Care for haematologic cancers

Unpredictable disease trajectory

Different languages between palliative care and haematologists

Unclear treatment goals / focus on healing

Hyper-Optimistic care / unclear boundaries between curative and palliative

Unawareness of goals of palliative care for haematologic cancers

BARRIERS AND CHALLENGES FOR PC INTEGRATION

Role of the oncologist: Challenges



Challenge:
assure both broad care AND high quality

BARRIERS AND CHALLENGES FOR PC INTEGRATION



Actions to support the extension and strengthening of services for palliative care:

- ◆ National policies that integrate evidence-based palliative services into the continuum of care
- ◆ Promote universal health coverage and essential medicines policies
- ◆ Education about palliative care to students in undergraduate medical and nursing schools
- ◆ Adequate access to controlled medicines
- ◆ Access to all aspects of palliative care, under the supervision of trained health care professionals, as appropriate
- ◆ Ethical guidance related to the provision of palliative care, in areas such as equitable access, respectful care and community involvement in policies and programs
- ◆ Working in partnership with different sectors to foster operational research in palliative care, including the development of cost-effective models of such care
- ◆ Committed leadership

BARRIERS AND CHALLENGES FOR PC INTEGRATION



Major indicators of integration of Palliative Care into Oncology

1. Presence of palliative care inpatient consultation team
2. Presence of palliative care outpatient clinic
3. Presence of interdisciplinary palliative care team
4. Routine symptom screening in the outpatient oncology clinic
5. Routine documentation of advance care plans in patients with advanced cancer
6. Early referral to palliative care
7. Proportion of outpatients with pain assessed on either of the last two visits before death
8. Proportion of patients with 2 or more emergency room visits in last 30 days of life (negative indicator)
9. Place of death consistent with patient's preference
10. Didactic palliative care curriculum for oncology fellows provided by palliative care teams
11. Continuing medical education in palliative care for attending oncologists
12. Combined palliative care and oncology educational activities for fellows/trainees
13. Oncology fellows have routine rotation in palliative care

SUMMARY



- ♦ Patients with incurable cancer present palliative care needs throughout the continuum of their disease
- ♦ There is increasingly robust level 1 evidence of the benefit of palliative care for patients and caregivers
- ♦ Oncologists must acquire core competencies to provide generalist palliative care
- ♦ Overlapping roles and unclear goals of care are among the barriers to integration



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